Portal HTN

liver has dual blood supply

hepatic artery	portal vein
20%	70%
oxygenated	non-oxygenated

* PH when portal venous pressure is
  - >12 mmHg
  - 10-12 mmHg borderline

* Why blood flow comming from intestine, other organs to portal vein inc ??
  - As Vasodilators as NO should be detoxified in liver that is not working well so VD remain in circulation inc blood flow to portal vein

* pre sinusoidal → schistomiasis
  sinusoidal → cirrhosis
  post sinusoidal → veno-occlusive dis

* post hepatic causes as :-
  restrictive pericarditis

* Portal systemic collateral most commonly seen in
  - piles - caput medosa
  - esophageal varices

when BP inc in portal vein blood starts to enter in veins that 're normally closed causing porto systemic collateral

JULY

SAT	SUN	MON	TUE	WED	THUR	FRI
					1	2
3	4	5	6	7	8	9
10	11	12	13	14	15	16
17	18	19	20	21	22	23
24/31	25	26	27	28	29	30

CamScanner

Friday الجمعة

30 July ٣٠ يوليه

١٠ صفر ٢٣ أبيب

* Doppler Ultrasound : to see whether the blood flows into or away from the liver.

In pts with Cirrhosis, portal HTN blood flows away from blood.

Managment

عشان ممكن الأوردة تختفي

2 large canulas "lifesaving"

- give IV fluids start with non-colloids if didn't respond give colloids
- draw blood sample → cross matching → give him blood
- Then start drugs to stabilize pts to make endoscopy

يوليه

السبت	الأحد	الاثنين	الثلاثاء	الأربعاء	الخميس	الجمعة
					١	٢
٣	٤	٥	٦	٧	٨	٩
١٠	١١	١٢	١٣	١٤	١٥	١٦
١٧	١٨	١٩	٢٠	٢١	٢٢	٢٣
٢٤	٢٥	٢٦	٢٧	٢٨	٢٩	٣٠

* Vassopressin rarely used as it not only cause splanchnic VC but also affect heart, cerebral vessels may cause ischemia

* Terlipress selective VC of splanchnic vessel not affecting heart or brain

* Sandostabin :-
works on smooth muscles inside vessels instead of spasm and VC it cause its relaxation.

Dose :-
Ampoule is 1mg
1/2 Amp is given if bleeding persist give the other 1/2.
then continue on 50 µg/hr for 3-5 days as long as pt is bleeding
لو عدل منظار والأمور تمام كفاية ٣ / ٥ / ١ يوم

JULY

SAT	SUN	MON	TUE	WED	THUR	FRI
					1	2
3	4	5	6	7	8	9
10	11	12	13	14	15	16
17	18	19	20	21	22	23
24/31	25	26	27	28	29	30

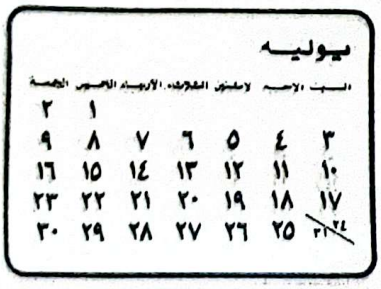
CamScanner

* Propranolol "for Prophylaxis"
Dose:- start with 40mg/day inc gradually every 3-5 days till HR reach 60 b/min

- HR shouldn't <60 if happened dec dose
- BP shouldn't be <90 mmHg

Carvid

* Nitrates
كان بيتكتب لمرضى الـ cardiac مع الـ vasopressin لكن بطلنا نستخدمه



الثلاثاء Tuesday

٢٧ يوليه ٧ صفر ٢٠ ابب July 27

* Portal HTN → bleeding varices → Spontaneous bacterial peritonitis (SBP) fatal if pt enter in an attack it inc risk of mortality

ممكن حد فيهم يدخل في الـ SBP عشان كدة بدي antibiotic prophylaxis

Ceftriaxone 1g/24 hr for 7 days if pt stayed in hospital for less than this period وبيخرج

give Oral Ciprocin 500mg (one tab) يكمل عليه

* صور الـ varices ممكن تيجي

لو لقيت المنظار باين ← Gastric

لو لقيت اسطوانة كدة ← Esophageal

* TIPS :

portal vein توصل الدم من stent بركب

لـ hepatic artery مشكلتها انها ممكن يحصل hepatic encephalopathy

ممكن تيجي

JULY

SAT	SUN	MON	TUE	WED	THUR	FRI
					1	2
3	4	5	6	7	8	9
10	11	12	13	14	15	16
17	18	19	20	21	22	23
24/31	25	26	27	28	29	30

CamScanner

Monday الإثنين

26 July ٦ صفر ١٩ ابيب ٢٦ يوليه

* Terlipressin , Saodostabin are used in :-
Bleeding esophageal Varices not stable pts

يوليه

السبت	الأحد	الاثنين	الثلاثاء	الأربعاء	الخميس	الجمعة
					١	٢
٣	٤	٥	٦	٧	٨	٩
١٠	١١	١٢	١٣	١٤	١٥	١٦
١٧	١٨	١٩	٢٠	٢١	٢٢	٢٣
٢٤/٣١	٢٥	٢٦	٢٧	٢٨	٢٩	٣٠

Ascites

الأحد Sunday

٢٥ يوليه July 25

٥ صفر ١٨ ابيب

Pathological Acc of fluid in peritoneal cavity

Other causes as :-
nephrotic syndrome

Fluid move from portal circulation inside peritoneal cavity

* Grade 1 :- cirrhotic, ascites detected only by US

Ascites to be seen clinically must be 1.5 Litre

* Triglycrides, Familial hyperlipidemia, ascites due to pancreatitis :→
Ascitic fluid شبه دهون اللمف

Lymphedema → Ascitic fluid Chylous

JULY

SAT	SUN	MON	TUE	WED	THUR	FRI
					1	2
3	4	5	6	7	8	9
10	11	12	13	14	15	16
17	18	19	20	21	22	23
24/31	25	26	27	28	29	30

CamScanner

Saturday السبت

24 July ٢٤ يوليه

ارض الواقع بنقول ال
Na 2g/day

* Main Cause of Ascites :-
Salt, water retention due to ↑↑ aldosterone as result of activation of Rennin angiotensin system.

First give Spironolactone → didn't respond → Add Furosemide → didn't respond → Refractory Ascites

- transplantation الحل الوحيد
- supportive الباقي

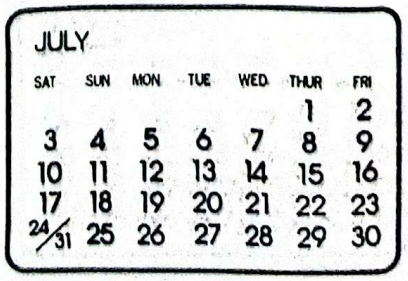


Furosamide work on :-
thick loop of henle

* Renal impairment, hepatic encephalop
→ stop diurekics

* Symptomatic or Asymptomatic SBP
PMN ≥ 250 /mm³
→ IV Cefotaxime 2g/8hr or Ceftriaxone 2g/24hr
repeat at 48hr if PMN dec by 25%
cont ttt for 5-7 days ,
if not responding switch to
Pipracillin_tazobactam
If didn't respond switch to
Ertapenem

اول ما بابدا ال antibiotic بسحب
acid blood culture
if (-) cont this ttt for 5-7 days
if (+) " " 2 weeks



CamScanner

Thursday الخميس

22 July ٢٢ يوليه

٢ صفر ١٥ أبيب

٨

* Probiotics ملهاش لازمة

٩

* طالما جه للعيان SBP attack يبقى لازم يكمل على SBP prophylaxis

١٠

١١

١٢

١

٢

٣

٤

٥

٦

٧

٨

يوليه

السبت	الأحد	الاثنين	الثلاثاء	الأربعاء	الخميس	الجمعة
					١	٢
٣	٤	٥	٦	٧	٨	٩
١٠	١١	١٢	١٣	١٤	١٥	١٦
١٧	١٨	١٩	٢٠	٢١	٢٢	٢٣
٢٤	٢٥	٢٦	٢٧	٢٨	٢٩	٣٠